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Ministry of Public Health, Hygiene and Social Welfare

National Institute of Public Health

Public Health Emergency Operations Center

PRESIDENTIAL EBOLA TASK FORCE 17

MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°175/MVE-BDBV/05/11/2026

Reporting date: November 5, 2026 Publication date: November 6, 2026

HIGHLIGHTS (last 24 hours)

Over the past 24 hours, sixty-two (62) new confirmed cases and 37 confirmed deaths, including 22 community deaths and 15 deaths within treatment centers, have been reported. The new cases were recorded in North Kivu (32), Ituri (28), Haut-Uélé (1), and Tshopo (1). A total of 454 new samples were analyzed in the laboratories that submitted their data; 62 of these samples tested positive for the Bundibugyo virus, representing a positivity rate of 13.7%. No new health zones were affected in the past 24 hours.

ACCUMULATIVE CASE	ACCUMULATIVE DEATH CONFIRMED	LETHALITY	PATIENTS IN ISOLATION/CTE	ACCUMULATIVE HEALED	TRACKING RATES CONTACTS (Of the Day)
10803	5267	48.8%	1020	2,912	82.6%

Affected provinces and affected health zones

8 Provinces affected

Haut-Uélé, Ituri, North Kivu, South Kivu, Tshopo, Bas-Uélé, South Ubangi, and North Ubangi

67 out of 178 health zones affected (37.6%)

Summary of key points from the last 24 hours

On November 5, 2026, sixty-two (62) new confirmed cases of Ebola virus disease (EVD) were reported in four provinces: North Kivu (32 cases), Ituri (28 cases), Haut-Uélé (1 case), and Tshopo (1 case), representing a 15.1% decrease compared to the previous day (73 cases). Among these cases, 22 community deaths were reported (13 in Ituri and 9 in North Kivu). In addition, 15 confirmed deaths were recorded in Ebola Treatment Centers (11 in Ituri and 4 in North Kivu), bringing the total number of confirmed deaths reported during this period to 37.

In addition, 24 patients (16 in North Kivu, 7 in Ituri and 1 in Haut-Uélé) were declared cured after obtaining two negative results in the control test for Ebola virus disease.

Since the start of the epidemic, the DRC has recorded 10,803 confirmed cases, including 5,267 deaths, representing an overall case fatality rate of 48.8%. Ituri remains the epicenter of the epidemic, accounting for 69.8% of cumulative confirmed cases and 45.2% of new reported cases, while North Kivu represents 51.6% of new cases and has a case fatality rate of 54.6%, higher than the national average.

No new health zones have been affected in the last 24 hours.

Thus, 67 out of 178 health zones (37.6%), spread across eight provinces, have been affected since the start of the epidemic, namely: Ituri, North Kivu, Haut-Uélé, Tshopo, South Kivu, Bas-Uélé, South Ubangi and North Ubangi.

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Table 1. Distribution of confirmed cases and deaths by affected province, as of November 5, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	28	7543	3,565	47.3%	28/36 (77.8%)
North Kivu	32	2661	1,454	54.6%	17/34 (50.0%)
Haut-Uélé	1	477	211	44.2%	7/13 (53.8%)
Tshopo	1	96	30	31.3%	7/23 (30.4%)
South Kivu	0	3	1	33.3%	1/34 (2.9%)
Bas-Uélé	0	13	4	30.8%	4/11 (36.4%)
South Ubangi	0	2	1	50.0%	1/16 (6.3%)
North Ubangi	0	8	1	12.5%	2/11 (18.2%)
Total	**62**	**10,803**	**5,267**	**48.8%**	**67/178 (37.6%)**

Table 2. Distribution of confirmed cases and deaths by province and zone as of November 5, 2026

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	**7543**	**3565**	**47.3%**	**28**	**13**	**11**	**24**
Adja	11	1	9.1%				0
Aru	25	7	28.0%				0
Ariwara	8	2	25.0%				0
Aungba	12	5	41.7%				0
Bamboo	220	50	22.7%				0
Boga	2	2	100.0%				0
Bunia	2031	715	35.2%	6	5	0	5
Damascus	41	10	24.4%				0
Drodro	15	7	46.7%				0
Fataki	84	31	36.9%				0
Gethy	11	7	63.6%				0
Kambala	2	0	0.0%				0
Kilo	40	13	32.5%				0
Komanda	193	119	61.7%				0
Lita	334	168	50.3%	1	1	0	1
Logo	14	9	64.3%				0
Lolwa	37	11	29.7%				0
Mahagi	6	1	16.7%				0
Mambasa	126	66	52.4%	2	0	1	1
Mandima	526	284	54.0%	11	3	2	5
Mangala	409	177	43.3%	1	1	2	3
Mongbwalu	728	355	48.8%	0	0	2	2
Nia-Nia	343	200	58.3%	0	0	1	1
Nizi	859	316	36.8%	3	3	0	3
Nyankunde	141	44	31.2%	1	0	1	1

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Rimba	28	7	25.0%				0
Rwampara	1211	438	36.2%	3	0	2	2
Tchomia	86	27	31.4%				0
To ventilate	N / A	493	N / A				0
North Kivu	**2661**	**1454**	**54.6%**	**32**	**9**	**4**	**13**
Blessed	797	511	64.1%	22	8	1	9
Good	26	10	38.5%	0	0	1	1
Butembo	347	203	58.5%	1	0	0	0
Goma	1	0	0.0%				0
Kalunguta	109	44	40.4%	5	0	0	0
Katwa	683	423	61.9%	0	0	1	1
Kayna	3	2	66.7%				0
Kyondo	46	22	47.8%				0
Lubero	43	16	37.2%	0	0	1	1
Mabalako	124	40	32.3%	2	1	0	1
Alimbongo	6	4	66.7%				0
Manguredjipa	16	5	31.3%				0
Masereka	48	14	29.2%	1	0	0	0
Musienene	163	79	48.5%				0
Mutwanga	88	22	25.0%	1	0	0	0
Oicha	103	39	37.9%				0
Vuhovi	58	20	34.5%				0
Upper Uele	**477**	**211**	**44.2%**	**1**	**0**	**0**	**0**
Boma Mangbetu	46	20	43.5%				0
Dungu	1	1	100.0%				0
Gombari	3	1	33.3%				0
Isiro	120	56	46.7%	1	0	0	0
Pawa	120	67	55.8%				0
Rungu	9	5	55.6%				0
Wamba	178	61	34.3%				0
Tshopo	**96**	**30**	**31.3%**	1	0	0	0
Bafwasende	18	3	16.7%				0
Kabondo	24	5	20.8%	1	0	0	0
Lubunga	1	0	0.0%				0
Makiso - Kisangani	37	14	37.8%				0
Mangobo	10	7	70.0%				0
Tshopo	5	0	0.0%				0
Wanie -Rukula	1	1	100.0%				0
South Kivu	**3**	**1**	**33.3%**	0	0	0	0
Miti-Murhesa	3	1	33.3%				0
Bas Uélé	**13**	**4**	**30.8%**	0	0	0	0
Buta	1	1	100.0%				0
Viadana	6	1	16.7%				0
Dakwa	1	0	0.0%				0
Ganga	5	2	40.0%				0
South Ubangi	**2**	**1**	**50.0%**	0	0	0	0
Bulu	2	1	50.0%				0
North Ubangi	**8**	**1**	**12.5%**	0	0	0	0
Wapinda	5	1	20.0%				0
Yakoma	3	0	0.0%				0
Total	**10803**	**5267**	**48.8%**	**62**	**22**	**15**	**37**

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*These are the confirmed deaths that occurred in the various CTEs in the province which are being broken down to be classified by health zone.

NA: Not Applicable.

Table 3. Status of alerts notified by province for November 5, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated ed	Number of suspects No investigated ed	Number of suspects investigated and transferred At CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased			
Haut-Uélé	110	1	111	14	0	96	1	14	0	5
Bas Uélé	104	0	104	6	0	98	0	6	0	0
Ituri	1,515	89	1,604	162	89	1,351	0	238	13	141
North Kivu	1,872	80	1952	197	80	1,675	0	277	0	133
South Ubangi ND		ND	ND	ND	ND	ND	ND	ND	ND	ND
North Ubangi	4	0	4	4	0	0	0	4	0	0
Tshopo	110	0	110	7	0	103	0	7	0	1
Total General	**3,715**	**170**	**3,885**	**390**	**169**	**3,323**	**1**	**546**	**13**	**280**

Figure 1. Evolution of cases by date of symptom onset over the last 21 days

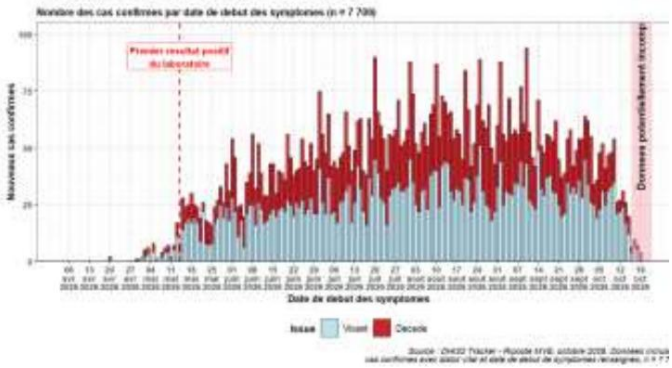
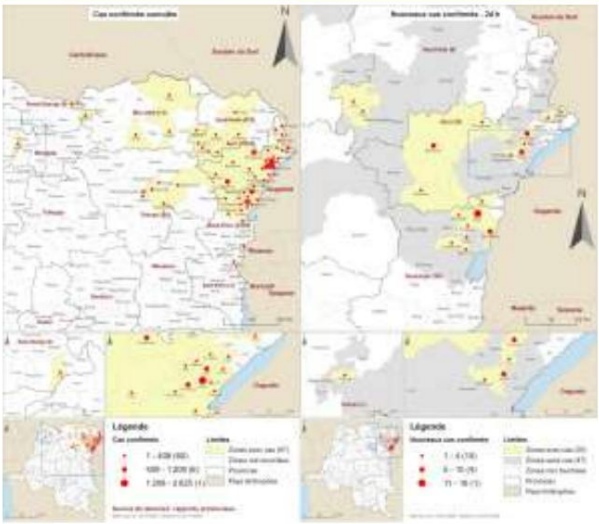


Figure 2. Spatial distribution of confirmed cases (24 hours and cumulative) by health zone.



1. COORDINATION

1.1. Main Actions

• Support for response activities in the provinces

• 2. RESPONSE ACTIONS BY PILLAR

2.1. Epidemiological surveillance

2.1.2. Main Actions

As of November 5, 2026, 3,885 alerts had been reported in the six provinces that submitted their data (Bas-Uélé, Haut-Uélé, Ituri, Nord-Kivu, Nord-Ubangi, and Tshopo), including 3,715 alerts of living persons and 170 of deaths; 3,883 were verified (99.9%). Among them, 559 (14.4%)

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were validated as suspected cases and 3,324 (85.6%) were invalidated. Of the 559 validated suspected cases, 546 were investigated (97.7%) and 280 were transferred to CT/CTE (Table 3).

As of November 5, 2026, a total of 38,054 contacts were under follow-up in the six provinces that reported contacts to follow up, of which 31,419 were seen in the last 24 hours, representing a follow-up rate of 82.6%, below the required threshold of 95%.

Table 4. Status of contact tracing by province, as of November 5, 2026

Province	Contacts under follow-up	Contacts viewed	tracking rate
Ituri	14,973	13,366	89.3%
Haut-Uélé	2,152	1,794	83.4%
North Kivu	19,262	15,182	78.8%
Tshopo	1068	492	46.1%
Bas-Uélé	0	0	N / A
South Ubangi	112	109	97.3%
North Ubangi	487	476	97.7%
Total	**38,054**	**31,419**	**82.6%**

2.2. Update on checkpoint and point of entry (PoE/PoC) activities

2.2.1. Main actions

For November 5, 2026, four (4) alerts were reported, two (2) in Ituri and two (2) in North Kivu. None were reported in South Kivu, Tshopo, or Bas-Uélé (Table 5). In Ituri, the two live alerts were validated and transferred. In North Kivu, at the Kasitu PoC (Beni), one live alert was validated and referred to the Beni General Referral Hospital (100%; 1/1), and the deceased body from Lukaya (Ituri) was intercepted, validated, and sent to the morgue for swab (100%; 1/1). In total, the 3 validated live alerts were transferred to CT/CTE (100%; 3/3), the intercepted body was swabed or secured (100%; 1/1) and the 4 alerts were investigated within 24 hours (100%; 4/4).

Table 5. Status of key PoE/PoC indicators by province, as of November 5, 2026

Indicators	**Ituri**	**North-Kivu**	**South Kivu**	**Tshopo**	**Bas-Uele**	**Overall**
Report completeness	92% (57/62)	95% (20/21)	56% (5/9)	100% (18/18)	50% (11/22)	84.1% (111/132)
Strategic PoC/PoE	1	N / A	N / A	N / A	N / A	1
Number of past travelers	147 103 95	793 7 173		23,989	2,345	276,403
Proportion of travelers screened	97%	99%	98%	100%	98%	98%
% of travelers who washed their hands	96%	99%	98%	99%	98%	97%
% of those travelers made aware	100%	99%	100%	100%	76%	99%
Number of alerts notified	02	02	00	00	00	4

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Number of validated live alerts	02	01	00	00	00	3
Number of lifeless bodies intercepted	00	01	00	00	00	1
% of suspected cases transferred to CT/CTE	100% (2/2)	100% (1/1)	0%	0%	0%	100% (3/3)
% of lifeless bodies swabed/secured	0%	100% (1/1)	0%	0%	0%	100% (1/1)
% of suspected cases investigated within 24 hours	100% (2/2)	100% (2/2)	0%	0%	0%	100% (4/4)

2.2.2. Challenges

In Bas-Uélé, the completeness is only 50% (11/22): 5 PoCs in the Viadana health zone did not work due to the strike of service providers and 6 did not report due to lack of connectivity and mobility, only 76% of travelers were sensitized and coverage remains at 22 sites out of 61 (36%); **in Ituri,** the completeness is 92% (57/62), 4,906 travelers (3.3%) refused screening, with high refusal rates in Nzebi (49%), Pluto (32%), Poipo (25%) and Frigo (24%), three PoCs in the Nia-Nia health zone did not work (claim for two months of payment), the one in Avakubi remains closed due to insecurity and the Bella PoC (Mandima) did not transmit its data; **In Haut-Uélé,** only 9 out of 12 PoCs reported, 620 travellers (3.9%) refused screening, including 440 in Bangwene (36%), and the Magambe PoC remains closed due to the strike by service providers; in Sud-Ubangi, the Libenge and Dongo posts did not transmit their reports (completeness: 60%; 3/5)

Laboratory

2.2.3. Main actions

- **Ituri:** 28 new positive results (15 living and 13 dead) out of 186 samples received and analyzed (positivity: 15.1%).

North **Kivu:** 32 new confirmed cases (23 living and 9 dead); laboratories in Beni and Butembo have received 279 samples and analyzed 232 of them (positivity: 13.8%)

- Tshopo: 1 new positive result (1 live) out of 6 samples received and analyzed (positivity: 16.7%).
- Haut-Uélé: 1 new positive result (1 live sample) out of 18 samples analyzed at the Isiro laboratory (positivity: 5.6%
- **Bas Uélé:** 12 samples (11 blood and 1 swab) from Bondo, Buta and Titule were analyzed at the laboratory of Buta and 15 others from Ganga and Viadana at the Isiro laboratory, all with negative results.

2.3. Infection Prevention and Control (IPC/EDS)

2.3.1. Main actions

- **Ituri:** 15 rings opened out of 29 (51.7%), 42 households decontaminated out of 52 (80.8%) and 5 FOSA out of 8 (62.5%); 69 EDS carried out out of 102 alerts, including 15 postponements (67.6%), 75 bodies swabés out of 84; the CREC facilitated the decontamination of 26 households out of 26 (100%), including 1 structure and 1 church, and of 5 ESS, the carrying out of 40 EDS out of 43 (93.0%) and the securing of 11 bodies; the teams remain insufficient in Mangala, Mandima, Damas and Mambasa.

North **Kivu:** 9 out of 16 rings opened (56.3%), 7 out of 19 households decontaminated (36.8%), none having received kits, and 16 out of 19 ESSs (84.2%); training and capacity building: 3rd day of the IPC-WASH training in an Ebola context for 28 providers from 6 ESSs in the health zones of Butembo and Katwa with CARE support; in schools and public places, 3 schools decontaminated and 4 assessed in Beni, 3 schools decontaminated in Katwa (Vingaze Institute, La Victoire Health Center and La Victoire Health Center) Thanks to this), 1 school equipped in Masereka, 43 people sensitized and 1 public place decontaminated in Kalunguta; 2 infected PPL out of 17 confirmed cases (11.8%); 64 EDS performed ，61 swabed bodies.

- **Tshopo:** 1 ring opened (Makiso-Kisangani) and decontamination of a household around the 94th case (Kabondo) and of a health center around the 95th case (Makiso-Kisangani), as well as of the inspection office and the main hall of the DPS; training and capacity building: completion of the training of 60 PPLs from DHKIS on IPC before its reopening, monitoring and support of 17 out of 17 health centers in 5 health zones with 216 providers briefed on standard precautions (100%), assessment of 3 high-risk PPLs from the Makiso General Referral Hospital around the 95th case and inventory of EVD transit sites in 5 health centers in Mangobo; cumulatively, 59 out of 107 health centers (55.1%) received IPC kits and 8 out of 107 (7.5%) have a score greater than 80%; 1 complete EDS made and 1 swabé body.

- **Haut-Uélé:** 4 rings opened in Pawa, 20 out of 20 households decontaminated (100%) and 6 out of 6 ESS (100%), including the Pawa CTE around confirmed PPL; training and strengthening of

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capabilities: 11 nurses briefed in Wamba on the functionality of DLM, safe injection and standard precautions, with provision of PCI kits and functional triage in the 4 ESS visited in the ZS; 4 EDS carried out on 4 alerts (100%) and 4 swabé bodies (Pawa, Boma Mangbetu and Wamba).

2.3.2. Bas-Uélé: monitoring and support of 11 out of 11 ESSs in the health zones of Buta (7) and Ganga (4); training and capacity building: briefing of 22 out of 22 service providers and 2 out of 2 hygiene workers (100%) on the mandatory wearing of PPE, waste management, sorting operation, hand washing and preparation of chlorinated solutions; awareness-raising in public places in Buta (Rawbank, central market and FARDC market).

2.3.3. Challenges.

In Bas **-Uélé,** only 130 out of 632 service providers (20.6%) are trained in IPC-WASH, 16 out of 20 priority health facilities (HHFs) lack functional triage, 17 out of 20 lack water for 72 hours, none of the 20 have mattresses, the available chlorine is limited to 45 kg, no WASH partners support activities around cases, and the service provider strike has intensified. **In Haut-Uélé,** opening the rings remains difficult in Isiro due to community resistance, only 4 out of 7 health zones have submitted their IPC data, IPC kits are lacking for HHFs, IPC supervisors are not activated in 4 health zones (Rungu, Dungu, Boma Mangbetu, and Gombari), and the EDS teams trained by FHI 360 are not yet operational. **In Ituri,** only 15 out of 29 rings (51.7%) could be opened, teams remain insufficient in Mangala, Mandima, Damas and Mambasa, IPC inputs are lacking in Mandima, Mambasa and Nia-Nia, the strike is disrupting activities in Mangala and 16 sampling failures were recorded on bodies.

2.4. Holistic care

2.4.1. Main actions

In Ituri, 96 new admissions (3 confirmed and 93 suspected, including 6 admitted in the advanced stage) were recorded, compared to 96 discharges, including 7 recoveries and 1 discharge without medical advice in Nizi. Thus , 344 patients (116 confirmed and 228 suspected) remained in isolation at the end of the day (341 the previous day), representing an overall occupancy rate of 34%; 329 out of 336 patients (97.9%) received hot meals, all children under LNPE/SLM were cared for (8/8 at the CTE, 4/4 in daycare, and 66/66 in the community), 31 patients were evacuated, and 18 out of 34 transfers (52.9%) were organized.

- **In North Kivu,** 106 new admissions (14 confirmed and 92 suspected) were recorded against 162 discharges, including 16 recoveries. In total, 578 patients (192 confirmed and 386 suspected, including 65 children under 5 years old and 25 critically ill) remain in isolation at the end of the day (595 the day before), including 184 outside CTE/CT, the CTE/CT being occupied at 90.8% (394/434 beds); 11 out of 17 ambulances (64.7%) carried out 19 trips, all successful (100%), for 21 calls received, 15 unstable confirmed patients benefited from specific diets, the 3 malnourished children and FEFA (100%) were taken care of and the 17 separated or orphaned non-breastfed infants are on SLM (100%).

- **In Tshopo,** 2 new admissions (1 confirmed and 1 suspected) were recorded against 1 discharge, the death of the biologist transferred from Nia-Nia; 10 patients (5 confirmed and 5 suspected, including 1 at CT Kongakonga) remain in isolation at the end of the day (9 the day before), representing an occupancy rate of 25.0% (10/40 beds), 1 patient being in serious condition; 5 confirmed, 5 suspected, 4 accompanying persons and 48 PPL received hot meals, the 3 available ambulances (100%) ensured 1 successful trip and, cumulatively, 66 confirmed received experimental treatment (63 on remdesivir and 3 on MBP134) and 601 people received post-exposure prophylaxis.

- **In Haut-Uélé,** 11 new admissions (1 confirmed in Isiro and 10 suspected, including 1 PPL in Pawa) were recorded against 9 discharges, including 1 recovery in Boma Mangbetu, 7 non-cases and 1 death of a suspected case in Wamba; 40 patients (14 confirmed and 26 suspected) remain in isolation at the end of the day (35 the day before), representing an occupancy rate of 24.5% (40/163 beds), none being in serious condition; 40 patients, 47 companions and 88 PPL received hot meals and 7 ambulance trips transported 7 patients.

In Bas **-Uélé,** four new admissions of suspected cases were recorded (three in Monga and one in Viadana), compared to 15 discharges of non-cases (eight in Viadana, four in Bili, one in Ango, one in Bondo, and one in Likati). Thirty-six suspected cases remain isolated in intensive care units (ESS) due to the lack of a functioning treatment center (CTE). In Nord-Ubangi, twelve patients (five confirmed and seven suspected) are isolated at the Wapinda General Referral Hospital (HGR) due to the lack of a CTE and are receiving nutritional and psychological support. No data on patient care was reported in Sud-Ubangi.

2.4.2. Challenges

In Ituri, the blood shortage persists, 37 patients are critical, 7 patients have not received food in Mandima, reception capacity remains low in Mambasa, Kilo, Lolwa, Mandima and Biakato and the Kasenyi CTE has still not reported due to the strike of service providers (completeness: 27/28) ;

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North Kivu, 184 patients are being treated outside of Ebola Treatment Centers (ETCs), ETCs are 90.8% occupied, 25 patients are critical (16 in Beni, 6 in Katwa and 3 in Oicha) and 3 deaths of suspected individuals have been recorded in Beni; **in Tshopo,** 2 confirmed escapees from Bafwasende remain missing, the house of a RECO (Reconnaissance and Coordination Officer) was burned down in Bafwasende and a group of young people attempted to recover a lifeless body; **in Bas-Uélé,** the standardized ETC in Buta is still under construction, no ambulance is available and none of the 36 isolated suspects are receiving nutritional support; **In Haut-Uélé,** only 2 out of 8 CTEs are standard, service providers are threatening to go on strike due to lack of payment, network coverage remains weak in some health zones and nutritional inputs (F75, F100, Plumpy'Nut) as well as the water and energy supply of CTEs remain insufficient; **in Nord-Ubangi,** no CTE is available in Wapinda and the inputs for care remain insufficient.

2.5. Vaccination

2.5.1. Main actions

- **In Ituri,** 194 people were vaccinated during the day (105 women, including 3 pregnant and 5 breastfeeding women, and 89 men), bringing the total to 3,023 people vaccinated (including 31 pregnant women and 89 breastfeeding women), with 41 non-severe AFI and 5 severe AFI reported since the beginning; vaccination of PPL and research on the effectiveness of Ervebo against BDBV Ebola virus disease continue. **In North Kivu,** 299 PPLs were vaccinated (149 at CS Kasabinyole, Beni Health Zone, and 150 at Kalunguta General Referral Hospital), bringing the total to 2,266 vaccinated (1,282 men and 984 women, including 16 breastfeeding women) in 4 Health Zones (Beni: 1,790; Oicha: 200; Kalunguta: 150; Mabalako: 126) out of 7,105 expected (31.9%), with 334 doses remaining out of 2,600 received, no doses lost and 1 non-serious AEFI reported.

In Haut **-Uélé,** 3,000 doses of Ervebo were received at the Isiro Expanded Program on Immunization (EPI) center, and members of parliament and army officers were briefed at the Isiro General Referral Hospital (HGR) vaccination site. **In Bas-Uélé,** the compassionate vaccination support team remains deployed in the Bili health zone while planning is underway for Bondo and Monga. Field workers have been paid, and the vaccination of motorcycle taxi drivers is being prepared. Cumulatively, 2,000 out of 13,202 targeted individuals (15.1%) have been vaccinated in the province. **In Nord-Ubangi,** 500 doses of Ervebo were received for the compassionate vaccination of people living with HIV (PLHIV) and people living with HIV (PLHIV), scheduled for November 9-14, 2026, following briefings in the eight targeted health zones.

2.5.2. Challenges

In Ituri, the TPL/PPL target remains to be redefined and 5 serious AFIs have been reported cumulatively; in North Kivu, vaccination remains concentrated in the Beni health zone (79.0% of those vaccinated), coverage reaches only 31.9% of the 7,105 people expected and there are only 334 doses left, or 12.8% of the stock received, hence the request for supplies addressed to the national coordination; in Bas-Uélé, the start-up of the CCUs is still not effective and the Bili health zone has not yet had any vaccinated individuals (0/1,067); in North Ubangi, vaccines still need to be made available in sufficient quantities.

2.6. Continuity of Care

2.6.1. Main actions • In North Kivu, the
assessment of the continuity of essential health services at the Mambowa General Referral Hospital (Biena) revealed the closure of the emergency department without any alternative, a non-functional 24-hour triage, weak protection of staff and 18 agents in quarantine without measures to maintain services, with only 11% of staff trained in IPC.

- **In Tshopo,** 3 ESS were evaluated (CM Béthanie and Clinique Familia, ZS of Tshopo, and CS Unikis of the Central Campus, ZS of Makiso-Kisangani): oxytocin is available in all three structures, but triage is not effective, none has an incinerator, the Central Campus has neither a garbage pit nor reception, consultation and laboratory registers, only 1 out of 13 providers is trained on Ebola at Clinique Familia (7%) and none at the Central Campus or at Béthanie, staff vaccination remains low (18% at the Central Campus, 23% at Familia and 38% at Béthanie) and, at Béthanie, 5 out of 20 children (25%) missed their CPS appointment and 8 out of 15 pregnant women (53%) missed their ANC appointment; The recovery plans for CM Mbuyu Butoke II and CS Lobiko have been developed and the training of managers and service providers on transfusion safety has continued (J3).

- **In Haut-Uélé,** the integrated supervision of the mosquito net distribution campaign continued in the health zones of Niangara and Dungu with the integration of Ebola messages and routine vaccination, and work on the recovery plan for the Ebola treatment centers of the Provincial Health Division began; in Bas-Uélé, the strike by nursing providers is affecting the entire province.

2.6.2. Challenges

Implementation of the recovery plans for social and solidarity economy (SSE) assessed in Tshopo is still pending, where the strike by public SSE employees and the non-activation of the PEV continuity plan persist, with a decline in utilization.

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curative services, IMCI, ANC, maternity and vaccination services; in North Kivu, the closure of the emergency department at Mambowa General Referral Hospital and the quarantine of 18 staff members without a service maintenance plan compromise access to care in Biena; in Haut-Uélé, the population deplores the insufficient quality of care offered in health facilities; in Bas-Uélé, the indefinite strike by nursing providers paralyzes the provision of care throughout the province; no data transmitted by Ituri, Sud-Ubangi and Nord-Ubangi.

2.7. Risk Communication and Community Engagement (RCCE)

2.7.1. Main actions

- **In Ituri,** 16,798 home visits reached 109,472 people, 275 educational talks reached 4,236 people, 13 community debates reached 288 people, 17 mass awareness campaigns reached 2,302 people and 6 advocacy campaigns reached 29 authorities, including religious authorities; 455 community alerts, including 209 deaths, were notified, 42 out of 47 feedback reports (89.4%) and 4 out of 7 incidents (57.1%) were resolved within 48 hours; 129 motorcycle taxi drivers from the Ndjili parking lot (Bunia) and 123 students from the Magagi Primary School (Lembabo School Association) were sensitized and the release of 3 recoveries was celebrated in Mambasa; In support of the other pillars, the CREC facilitated the pre-listing of 1,258 contacts around 9 confirmed cases, the decontamination of 26 out of 26 households (100%), including 1 structure and 1 church, and 5 ESS, the completion of 40 out of 43 EDS (93.0%) and the securing of 11 bodies, the announcement of 21 positive results out of 22 and the transfer of 55 out of 61 suspects (90.2%) to CTEs; only 11 out of 28 ZS (39.3%) reported.

- **In North Kivu,** 34,748 home visits reached 119,978 people, 169 educational talks reached 7,350 people, 85 mass communication sessions reached 6,223 people and 6 briefings reached 533 people; 582 out of 650 expected alerts (89.5%) were notified, 23 out of 34 feedbacks (67.6%) and 1 out of 7 incidents (14.3%) were resolved within 48 hours and 346 out of 4,750 people (7.3%) were engaged in community actions; in Beni, the 2nd day of updating the CREC plan was devoted to cross-functional work; In Butembo, following a plea, the pastor in charge of the Pilgrims Without Borders church pledged to deny his messages against prevention measures, and the release of 4 recovered and 5 non-cases was accompanied by a media testimony on the importance of early transfer to CT/CTE; 300 students from the Buyori Institute (Mutwanga) and more than 367 schoolchildren and 15 teachers from the Mangodomu Primary School (Mabalako) were sensitized, and the CREC facilitated the listing of 56 contacts and the decontamination of 4 households and 2 structures around the latest cases in Butembo, Mabalako, Vuhovi and Katwa.

- **In Haut-Uélé,** 322 home visits reached 2,254 people, 10 educational talks reached 110 people and 9 mass activities reached 2,225 people, for a total of 4,589 people sensitized; 8 community alerts were notified (6 in Wamba and 2 in Isiro); a meeting with APAs and community leaders, chaired by the deputy mayor of Isiro, focused on intensifying awareness in markets and transport agencies, a dialogue was facilitated with young people suspected of causing disturbances in Bolebole, who pledged to support the response, and 12 people linked to 2 suspected community cases were sensitized to contact tracing, decontamination and EDS in the Gossamu health area; CREC also supported the decontamination of CH Mayogo and 2 households and supervised the integration of Ebola messages into the distribution of mosquito nets.

- **In Tshopo,** 210 home visits reached 3,645 people, 4 educational talks reached 421 people and 10 mass awareness campaigns reached 5,508 people (Lubunga, Kabondo, Makiso, Bafwasende and Tshopo); 48 community alerts were notified (26 in Makiso, 18 in Tshopo and 4 in Kabondo) and 11 feedbacks out of 11 (100%) were resolved within 48 hours; 25 RECOs from the Makiso-Kisangani Health Zone were trained with the support of the project “Acting today for a future without an Ebola epidemic”, the mapping of CREC’s PTFs was updated and a handwashing demonstration was organized in Kabondo schools in support of IPC.

In Bas **-Uélé,** 486 home visits reached 2,501 people, one educational talk reached 23 people, two mass awareness campaigns reached 435 people, and two radio programs reached approximately 25,985 listeners in Buta. Twenty-three students from the Dingila Technical Institute (ITM) were briefed on Ebola Virus Disease (EVD) and community engagement, and an awareness campaign was conducted at the opening of the academic year at the Free Protestant University in Africa. No community alerts were reported, and only 4 out of 13 health zones (ZS) submitted reports. In Nord-Ubangi, the CREC subcommittee met, and the dissemination of awareness messages continues on local radio stations.

2.8. Mental Health and Psychosocial Support (MHPSS)

2.8.1. Main actions

In Ituri, 25 of the 30 confirmed cases (83.3%), 68 confirmed cases out of 91 under follow-up, 43 new suspected cases out of 96, and 126 suspected cases out of 132 under follow-up have been treated; 16 out of 19 (84.2%) households with separated and orphaned children have received support; 88 children are being monitored, including 22 out of 22 in daycare with 4 new admissions and 1 family reunification; 5 out of 7 have recovered and been reintegrated, and 28 out of 33 are being monitored in the community; and 95

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Results announced, including 25 positive ones; 247 caregivers and 77 PPLs were supported and the capacities of Psychologists and APS/TPS in Rimba, Logo and Mahagi have been reinforced.

- **In North Kivu,** 8 out of 9 notifying health zones were covered, 42 of the last 52 confirmed cases (80.8%), 216 confirmed cases under follow-up, 116 new suspected cases and 165 suspected cases under follow-up were taken care of, 32 out of 41 (78.0%) separated and orphaned child households were supported, 5 out of 5 children were followed up in daycare and 28 out of 36 (77.8%) in the community, 16 out of 16 followed up in the community were cured, 104 results were announced, including 52 positive results, 549 people (183 men, 267 women and 99 children) benefited from psychoeducation (UNICEF support), 89 households were supported for public health actions and 68 PPLs were supported; Training for psychologists, community health workers, and community mobilizers on SMSPS support for recoveries was provided in Oicha (Eringeti health district). In Haut-Uélé, no SMSPS data was transmitted for the day.

In Tshopo, 6 out of 7 reporting health zones were covered, all 5 confirmed cases under observation at the Meteorological Treatment Center (5/5) received psychological support, the new suspected case of the day and the 3 suspected cases under observation were treated (100%), 1 negative result was announced, 33 caregivers and 1 person receiving psychoeducation on barrier gestures, 2 households received support, the removal of resistance at the Rekapi II morgue continues, and the recovered patient being monitored in the community was seen (1/1). In Bas-Uélé, no SMSPS interventions were reported for the day. In Nord-Ubangi, the 12 patients isolated at the Wapinda General Referral Hospital are receiving psychological support.

2.8.2. Challenges

2.9. In Ituri, only 15 out of 28 affected health zones (53.6%) are covered, 5 of the last 30 confirmed cases (16.7%) and 23 of the 91 confirmed cases currently being monitored have not received care, and only 43 out of 96 new suspected cases (44.8%) have received support. In North Kivu, 10 of the last 52 cases (19.2%) have not received support, coverage remains low around the last cases in Biena, Manguredjipa, Kayna, Lubero, Masereka, Kalunguta, Mabalako, Mutwanga, and Vuhovi, 10 affected health zones do not have a daycare center, and 9 out of 41 households with separated or orphaned children remain without support. In Haut-Uélé, there are no daycare centers in Isiro, Pawa, Wamba, or Boma Mangbetu, and community health workers lack transportation and nutritional kits for weaned infants. In Tshopo, community health workers in the Wanie-Rukula health zone are not trained, and the local health center lacks communication equipment and motorcycles. In Bas-Uélé, psychosocial support for the three confirmed cases is still lagging.

2.10. Logistics

2.10.1. Main actions

- **In Ituri,** 31 patients were evacuated and 18 out of 34 transfers (52.9%) were organized by ambulances, but reporting of ambulance movements remains low; a medical evacuation (MEDEVAC) of a confirmed PPL from Butembo was carried out by private jet from Bunia airport to the United Kingdom; for health products and medicines (HPM), the blood shortage persists and IPC medicines and inputs are lacking in Mandima and Mambasa, where teams are also waiting for motorcycles, vehicles, fuel and an ambulance to serve the checkpoints.

- **In North Kivu,** 11 out of 17 ambulances (64.7%) were mobilized for 19 trips, all successful, out of 21 calls received (response capacity: 61%); supplies: handwashing kits delivered to the Mangodomu Primary School (Mabalako); for the PSM, 2,600 doses of Ervebo have been received in total (2,266 used, 334 remaining, none lost) and a new supply of vaccines has been requested from the national coordination; the construction of huts and the passenger walkway at Beni airport is planned with the support of the IOM.

- **In Tshopo,** the Cinquantenaire site was prepared for PEP (erection of tents), a room at the Makiso General Referral Hospital was prepared for the isolation of service providers exposed during post-mortem cesarean section, the delivery of materials began for the rehabilitation of the Makiso isolation and the extension of the standardized CTE of Météo, and the Kongakonga isolation site was supplied with medical inputs after updating stocks; the mobility of pillars and the transport of participants to the intra-action review were ensured and the 3 available ambulances were mobilized (100%); for the PSM, the laboratory is facing a reagent shortage (MVE buffer), the breakdown of two modules of the Radione platform and a near-shortage of diesel (1,000 liters needed for the incinerator), and the mobility need of the ZS remains estimated at 115 motorcycles.

- **In Haut-Uélé,** a new Toyota Land Cruiser vehicle was provided by the central government, through the INSP, for the mobility of the response, the ambulance of the ZS of Isiro was repaired and 7 ambulance trips transported 7 patients; for the PSM, medicines and inputs for care were delivered to the CTE of Isiro and 3,000 doses of Ervebo were received at the PEV antenna of Isiro, the fuel stock went from 988 to 928 liters and there is a lack of fuel for the generator of the ultra-low temperature cold chain, while the disappearance of hydroalcoholic gels is reported.

In Bas -Uélé, the Bondo health zone has been supplied with EDTA tubes and PCI supplies; the packaging of PCI and PEC supplies for the health zones is underway; the rehabilitation of the meeting room and the DPS offices is progressing.

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Work continues with the WHO, and construction of the Ebola Treatment Center (ETC) at the Buta General Referral Hospital (HGR) has reached 50% completion. For the Public Health Units (PSM), the chlorine stock is limited to 45 kg, the laboratory lacks a fire extinguisher, an uninterruptible power supply (UPS), and a computer; no diesel fuel is available, and the 20 priority health centers (ESS) lack mattresses. In North Ubangi, four motorcycles have been deployed to the two affected health zones, and a shipment of WHO supplies has been received in Wapinda, but the ETC mattresses and other supplies are still pending.

2.11. Security

2.11.1. Main actions

- **In Ituri,** the Avakubi PoC remains non-functional due to insecurity; in North Kivu, a grenade was discovered in front of the entrance to the Beni Health Zone office, EDS teams were accompanied and secured for the burial of 2 confirmed cases at the Bundji cemetery and for swabs in Butuhe and the Vighole neighborhood, the IPC team was accompanied for the decontamination of a household in Manziko (Benengule Health Area) and the threat against PoC providers on the Butembo axis persists; **in Haut-Uélé,** the security threat against PoC providers persists on the Bolebole axis (Pawa and Wamba); in Tshopo, the house of a RECO was burned down in Bafwasende, where response activities remain suspended, and young people attempted to recover a lifeless body; In Bas-Uélé, the pillar assessed the security situation, secured the response and monitored activities at the zonal level.

2.12. PSEA (Prevention of Sexual Exploitation and Abuse)

2.12.1. Main actions

In Ituri, 2 members of the INRB CREC team (1 woman and 1 man) were briefed on the PSEAH and signed the code of good conduct, 22 wood sellers (12 women, 7 men and 3 boys) were sensitized in the Bunia health zone and the display of messages and the distribution of leaflets continue in the health zones of Bunia, Aru, Mongbwalu, Mambasa, Fataki and Mandima;

In North Kivu, 304 people were sensitized or briefed during 4 activities, including 6 focal points of WAA briefed in the AS of Murara (Karisimbi), 285 participants sensitized in Butembo with the support of the WHO and 13 people in Beni, 146 leaflets and 4 posters distributed and the toll-free numbers 495555 and 151 disseminated;

In Haut-Uélé, 9 PPLs from the Anualité hospital in Matali (5 men and 4 women, Wamba health zone) were sensitized on the PSEA, referral mechanisms and the rights of victims; in Bas-Uélé, 23 learners from the ITM in Dingila (17 boys and 6 girls) were sensitized on the PRSEAH.

2.12.2. Challenges

- Coverage of the affected areas remains limited: only Ituri, North Kivu, Haut-Uélé and Bas-Uélé have reported their PSEA activities for the day (4 out of 8 DPS; 50.0%); in Bas-Uélé, 4 ZS (Viadana, Bondo, Likati and Ango) remain silent, funding for the pillar is low and posters and banners are out of stock; the reporting mechanism is known in only 15 out of 36 ZS (41.7%) in Ituri and 6 out of 13 ZS (46.2%) in Haut-Uélé, only one partner (WHO) reports its PSEA activities in Haut-Uélé, some partners refuse to align in Ituri and awareness tools in French and Lingala, visibility materials and computer kits for the pillar remain insufficient.

KEY MESSAGES

Sixty-two (62) new confirmed cases were reported in five provinces, including 32 in North Kivu, 28 in Ituri, and 1 in Haut-Uélé. (Isiro), 1 at Tshopo (Kabondo). The transmission remains particularly marked in the health zones of Beni (22 cases) and of Kalunguta (5 cases) in North Kivu and in those of Mandima (11 cases) and Bunia (6 cases) in Ituri.

Of the 37 confirmed deaths reported, 22 are deaths Community cases (13 in Ituri and 9 in North Kivu) and 15 are deaths at the CTEs (11 in Ituri and 4 in North Kivu). The contact tracing rate stands at 82.6% (31,419/38,054) in the six provinces with contacts currently being monitored (North Ubangi: 97.7%; South Ubangi: 97.3%) %; Ituri: 89.3%; Haut-Uélé: 83.4%; North Kivu: 78.8%; Tshopo : 46.1%), a slight decrease compared to the previous day (82.7%) and still below the 95% threshold, with the lowest rate in Tshopo, where Data from Bafwasende is not available due to

TIMELINE OF KEY EVENTS

From April 24 to May 15, 2026, the epidemic is moving beyond the detection of the first cases suspected of having the virus confirmed by biological testing Ebola Bundibugyo, leading to the declaration official of the 17th epidemic of Virus Disease Ebola in the Democratic Republic of Congo.

Between May 17 and 18, 2026, the authorities national, with the support of partners, have strengthened the response with the declaration of the public health emergency of scope national and then continental, accompanied by

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